BASELINE → TREATMENT → FOLLOW-UP

Checks across the whole treatment course

The columns show the order of care, not equal lengths of time. Confirm who books each check, who reviews it and whom to call if it is missing.

Check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver	Baseline bloods; dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Neuropathy / cold sensitivity	Record existing nerve symptoms	Check persistence and effect on daily tasks	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review
Heart checks	Review cardiac history; electrocardiogram (ECG) when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00209 v10a · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Modified FOLFOX-6: treatment and checks

Gastrointestinal cancers

Day 1: oxaliplatin, folinic acid and fluorouracil, followed by a 46-hour fluorouracil pump. A cycle lasts 14 days.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Blood count / kidneys / liver NCCP 00209	Baseline bloods; DPD before fluoropyrimidine	Repeat before each cycle	Earlier tests if unwell, dehydrated or toxicity develops	Results and recovery guide any delay or dose change	Follow unresolved abnormalities
Symptoms and function NCCP 00209	Document existing symptoms and usual activity	Review worst symptoms since the last treatment	Use urgent contacts; do not wait for the next visit	Balance benefit, side effects and priorities	Agree recovery care and ongoing support
Neuropathy / cold sensitivity NCCP 00209	Record existing nerve symptoms	Check persistence and effect on daily tasks	Report worsening numbness or cold-triggered throat symptoms	Consultant reviews the oxaliplatin component separately	Nerve symptoms can continue after treatment; arrange review

Care / check	Before starting	Each treatment cycle	Between doses	Decision / response review	Completion / change
Heart checks NCCP 00209	Review cardiac history; ECG when cardiac function is compromised or clinical findings warrant it. No automatic serial echocardiogram schedule is specified for this regimen.	Ask about new chest pain or breathlessness	Chest pain needs urgent assessment	Repeat heart tests if clinically indicated	Follow any heart problem; no fixed echo series
Response / scans NCCP 00209	Confirm cancer setting, disease assessment and intended duration	Review clinical benefit and symptoms	New symptoms may bring review forward	Consultant-directed scans and response assessment	Agree follow-up, completion or another treatment

What each check is for

1 Confirm your personal plan

Before starting

Modified FOLFOX-6 combines oxaliplatin, folinic acid and fluorouracil. It can be used after colon-cancer surgery or to treat advanced colorectal, stomach or gastro-oesophageal-junction cancer. The aim and duration depend on the indication; ask which applies to you.

Ask / act: Agree medicines, treatment days, duration and monitoring.

Sources: NCCP 00209

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-04 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.32